

Patient 12

Report A

6524526 08/06/2016 CT Acute Stroke

Consultant addendum: I agree with the provisional report. In addition, there is a tandem stenosis involving the right M172 junction and Sylvian M2 branch, accompanied by two areas of poststenotic aneurysmal dilatation of the vessel. There is fusiform dilatation of the basilar tip. In addition to the significant atheromatous disease of the extracranial arteries, these findings are in keeping with intracranial vasculopathy.

For Thrombolysis? Yes

CT HEAD

Haemorrhage: No

Intravascular thrombus: No

Acute infarct: No Other findings and comments: Established right lenticulostriate and pontine infarcts. There is circumferential non-stenosing cavernous/supraclinoid ICA atheroma. There is mixed proximal intradural vertebral artery and basilar atheroma.

CT ANGIOGRAM

Window settings: (intraluminal HU=220; W=440; L=165)

Aortic arch and subclavian arteries: Moderate mixed volume arch atheroma with minor extension into the left subclavian origin. Left-sided aortic arch with bovine configuration and subclavian origins of the vertebral arteries.

RCCA/ICA: There is moderate right carotid bifurcation mixed atheroma causing eccentric narrowing of the proximal right internal carotid artery causing approximately 60% focal stenosis.

LCCA/ICA: Moderate volume left carotid bifurcation atheroma causing mild eccentric narrowing, no significant stenosis.

RVA. Minor ostial calcified atheroma. There is proximal intradural atheroma causing moderate narrowing at the level of the hypoglossal nerve canal, the vertebral artery remains patent.

L VA: Mixed intradural atheroma causes mild narrowing, no significant stenosis. Intracranial Arteries: Proximal branches at the circle of Willis are patent with no focal stenosis, occlusion or acute dissection. The posterior communicating arteries are not visualised. Other findings and comments: There is a biapical pleuro-parenchymal scarring with calcified right apical granulomata.

Conclusion:

1. No acute infarct.
2. Established right lenticulostriate and left hemipontine infarct.
3. Approximately 60% focal eccentric narrowing of the proximal right internal carotid artery,

Report B

6525400 09/06/2016 CT head

Clinical information:

24 hour CT head-admitted with RSW and facial droop. Thrombolysed

Findings:

Comparison is made with the CT head dated 08/06/2016. There is now a small area of low density in the left medial occipital lobe, in keeping with an evolving left PCA territory infarct. The remainder of the intracranial appearances are stable. In particular, no haemorrhage is shown. A background of moderate small vessel disease is again noted.